- Generally accepted standards of medical practice are based on credible scientific evidence published in peer-reviewed medical literature generally recognized by the relevant medical community, nationally recognized evidence-based clinical guidelines and criteria from independent sources, national physician specialty society recommendations and the views of medical practitioners practicing in relevant clinical areas, and any other relevant factors. Please refer to geha.com/Coverage-Criteria for details on the literature used in medical decision making.
- Are not primarily for the personal comfort or convenience of the patient, the family, or the provider;
- Are not a part of or associated with the scholastic education or vocational training of the patient;
- Are not more costly than an alternate drug, service(s), or supply that is at least as likely to produce equivalent therapeutic or diagnostic results for the diagnosis or treatment of your condition;
- In the case of inpatient care, cannot be provided safely as observation or on an outpatient basis;
- Is not custodial or long-term care (see the Plan's definition).

The fact that a covered provider has prescribed, recommended, or approved a service, supply, drug or equipment does not, in itself, make it medically necessary. We reserve the right to consult expert opinion in determining whether healthcare services are Medically Necessary.

Medicare Part A Part A helps cover inpatient hospital stays, skilled nursing facility care, hospice care, and some home health care.

Medicare Part B Part B covers medically necessary services like doctors' services and tests, outpatient care, home health services, durable medical equipment, and other medical services.

Medicare Part C Part C is a Medicare Advantage plan that combines the coverage of Medicare Part A and Part B. Part C typically also covers additional benefits like dental, vision, and hearing services. Part C plans also include Medicare Part D coverage.

Medicare Part D Medicare Part D plans provide coverage for prescription drugs. Private insurers contract with CMS on an annual basis for the right to offer Part D plans. Part D can be offered as a standalone Prescription Drug Plan (PDP) or as part of a Medicare Advantage Prescription Drug Plan (MAPD).

Mental health/ substance use disorder Conditions and diseases listed in the most recent edition of the International Classification of Diseases (ICD) as psychoses, neurotic disorders, or personality disorders; other nonpsychotic mental disorders listed in the ICD, to be determined by the Plan; or disorders listed in the ICD requiring treatment for use or dependence upon substances such as alcohol, narcotics, or hallucinogens; may also be collectively referred to as Behavioral Health conditions. Prior authorization is required for all of the following services and must be provided by a covered facility or covered provider as defined in Section 3, *How You Get Care.*

Inpatient Behavioral Health (includes mental health and substance use disorders):

- Acute Care Hospital: See Section 3, *Covered Facilities.*
- Residential Treatment Center (RTC): See Section 3, *Covered Facilities.*

Intensive Day Treatment:

Intensive day treatment programs are outpatient services that must be rendered on an outpatient basis. Regardless of where services are rendered, the provider and/or the facility, must be licensed to provide intensive day mental health and/or substance use treatment and must meet G.E.H.A's definition of a covered provider in Section 3. G.E.H.A does not cover room and board during intensive day treatment programs. Under the direction of a physician, services must be provided for at least two hours per day and may include group, individual, and family therapy along with psychoeducational services and adjunctive psychiatric medication management.